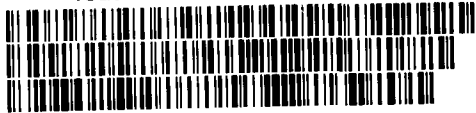


Redacted



Gender: Male

DOB:

Race:

Report Date:

Pathology Report:

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
IHPAA Discrepancy		
Prior Malignancy History		
Local/Synchronous Primary Tumor		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed Initials	W 4/2/12	4/2/12

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

Cystoprostatectomy with bilateral pelvic lymph node dissection,

"Bladder, prostate, vas deferens" (part C):

- Invasive high-grade urothelial carcinoma, see parameters below.
- Benign prostate and seminal vesicles.

"Right pelvic lymph nodes" (part A):

- Three of nine lymph nodes are positive for metastatic carcinoma (the largest implant almost completely replaces a 0.8 cm node).

"Left pelvic lymph nodes" (part B):

- Seven lymph nodes, no evidence of tumor (0/7).

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Flat urothelial carcinoma
2. Grade of tumor: High-grade
3. Depth of invasion: Tumor shows invasion to the outer half of the muscularis propria
4. Tumor distribution: Large solitary lesion involving the posterior wall, right lateral wall and left lateral wall as lesion measuring 5 x 3 cm grossly
5. Ureteral margins: Negative for tumor
6. Distal urethral margin: Negative for tumor
7. Soft tissue margin: Negative for tumor
8. Lymph nodes: Positive for tumor (see above parts A and B)
9. pTNM: pT2b, N2, MX

Effective this Checklist utilizes the edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

xx

[], MD

Interpretation performed by the Attending Pathologist.

Electronically Signed Out by [], MD

ICD-O-3
 carcinoma, urothelial, NOS
 8120/3

Site:

path: bladder wall, NOS

C67.9

COCF: bladder, wall,
lateral
C67.2

49-12
RD

Clinical History

The patient is a -year-old male with bladder cancer undergoing radical cystectomy, bilateral pelvic lymph node dissection, ileal conduit.

Specimens Received:

A: Right pelvic lymph node

B: Left pelvic lymph node

C: Bladder, prostate, vas deferens; cystoprostatectomy

Gross Description:

The specimens are received in three containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "right pelvic lymph node". Received fresh and placed in formalin is a 6.5 x 5.0 x 1.8 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal 9 apparent lymph nodes ranging from 0.3 x 0.2 x 0.1 to 6.1 x 1.1 x 1.0 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

A1-A2: 5 lymph node candidates
A3: One lymph node, bisected
A4: One lymph node, bisected
A5: One lymph node, bisected
A6-A7: One lymph node, sectioned

B. The second container is additionally identified as, "left pelvic lymph node". Received fresh and placed in formalin is a 6.7 x 4.5 x 2.0 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal 8 apparent lymph nodes ranging from 0.5 x 0.2 x 0.1 to 2.7 x 1.5 x 0.8 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

B1: 3 lymph node candidates
B2: One lymph node, bisected
B3: One lymph node, bisected
B4: One lymph node, bisected
B5: One lymph node, bisected
B6-B7: One lymph node, sectioned

C. The third container is additionally identified as, "bladder, prostate, vas deferens". Received fresh and placed in formalin is a 396.8 gm, 15.0 x 12.0 x 4.5 cm cystoprostatectomy specimen with attached mesenteric fat. The bladder measures 9.5 x 8.0 x 4.0 cm. The bilateral ureteral orifices are identified and probe patent. The right ureteral stump measures 4.5 x 0.4 cm and the left 4.5 x 0.5 cm, and both demonstrate patent lumina. The prostate measures 4.5 x 4.0 x 3.5 cm. The right half of the prostate and bladder is inked blue and the left half is inked black. The prostate is opened anteriorly along the urethra and continued superiorly through the bladder in a Y-shaped incision. The prostate is serially sectioned from apex to base to reveal unremarkable, bulging yellow-tan parenchyma without indurations; there is periurethral nodularity. The bladder demonstrates an area of ulceration with underlying induration on the right posterolateral wall that measures 5.0 x 3.0 cm. The mucosal surface overlying this induration has a flattened, granular appearance. Serial sectioning reveals

tumor invasion to within 0.2 cm of the inked margin, and grossly apparent invasion into the muscularis propria. Adjacent to this area of ulceration and induration, toward the left posterolateral wall, there is a poorly defined area of mucosa with a granular appearance without prominent papillary excrescences; the tissue underlying this granular mucosa is somewhat indurated. The remainder of the mucosa is tan-brown, edematous, and glistening with a uniform 2.5 cm wall thickness. The external surface of the bladder is red-tan and glistening with no grossly identifiable lymph nodes. Representative sections are submitted as follows:

- C1: Distal urethral margin
- C2: Prostatic apical margin
- C3: Right ureteral margin
- C4: Left ureteral margin
- C5: Right ureteral orifice
- C6: Left ureteral orifice
- C7-C12: Multiple tumor sections, including deepest invasion in C12
- C13: Representative section from adjacent, granular mucosa
- C14-C22: Representative sections of prostate from apex to base
- C23: Seminal vessels and vasa deferentia
- C24: Uninvolved dome
- C25: Uninvolved anterior wall
- C26: Uninvolved posterior wall
- C27: Uninvolved right wall
- C28: Uninvolved left wall

Gross photograph was taken of a representative section of the ulcerated, indurated region.

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[] M.D.